

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 79 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: February 10, 2025 | Provider: Eleanor Park, MD — Internal Medicine | Doc ID: RFH-2025-0210-CGA

Comprehensive Geriatric Assessment

Annual Multimorbidity Review & Polypharmacy Reconciliation (Age 79)

Subjective

79-year-old man for annual comprehensive review. Lives with spouse; independent in basic ADLs, needs help with heavy chores and shopping (instrumental ADLs). Uses a cane outdoors. One fall in past year (Jan 2023 wrist fracture). Reports good days and bad days with breathlessness. Mood generally stable; mild memory complaints (misplacing items). Goals: maintain independence, avoid hospitalization.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
118/70	66 irreg	16	36.5 C	96% RA	91.0 kg	29.7

Functional & Cognitive Screen

Gait/balance: mildly impaired, Timed-Up-and-Go ~14 s (elevated falls risk). Cognition: Mini-Cog 4/5 (borderline) — formal testing offered. Mood (PHQ-2) negative. Nutrition adequate; weight stable. Hearing mild loss; vision improved post-cataract surgery (OD).

Annual Surveillance Labs

Test	Result	Units	Reference Range	Flag
Hemoglobin A1c	7.5	%	< 8.0 (relaxed, geriatric)	
Creatinine	2.1	mg/dL	0.7 - 1.3	H
eGFR (CKD-EPI)	31	mL/min/1.73 m2	> 60	L
Potassium	4.8	mmol/L	3.5 - 5.1	
Hemoglobin	10.9	g/dL	13.5 - 17.5	L
LDL cholesterol	55	mg/dL	< 55	
PSA, total	0.4	ng/mL	Stable	

- Active Problem List**
- HFrEF (EF 30%), NYHA II-III — ICD evaluation in progress.
 - Atrial fibrillation — anticoagulated (apixaban).
 - CAD s/p LAD drug-eluting stent (2012).
 - CKD stage G3b/G4 (eGFR 31).
 - Type 2 diabetes with kidney disease, retinopathy, early neuropathy.
 - Prostate cancer — long-term remission (annual PSA).
 - COPD (GOLD 2).
 - Osteoporosis with prior fragility fracture (on denosumab).
 - Anemia (CKD + iron, treated).
 - Hypertension; dyslipidemia.
 - BPH; bilateral knee OA s/p right TKA; cataract s/p right IOL; prior TIA with left ICA stenosis.

Polypharmacy & Plan

• Medication reconciliation completed (13 chronic medications). Deprescribing review: relaxed A1c target (avoid hypoglycemia); metformin reduced for CKD; confirm denosumab calcium co-administration; ensure no NSAIDs.

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- Falls: PT referral, home safety evaluation, review for orthostasis; vitamin D continued.
- Cognition: offer formal cognitive assessment; review anticholinergic burden (low).
- Advance care planning discussed; documented preferences; spouse involved.
- Vaccinations updated (influenza, COVID booster, pneumococcal, RSV, shingles per age).
- Continue specialist co-management (cardiology, nephrology, oncology, pulmonology).

Reconciled Medication List (Chronic)

Medication	Dose	Route	Frequency	Indication
Sacubitril/valsartan	49/51 mg	PO	Twice daily	HFrEF
Carvedilol	12.5 mg	PO	Twice daily	HFrEF / AF
Spironolactone	25 mg	PO	Once daily	HFrEF (monitor K)
Furosemide	40 mg	PO	Once daily PRN	Congestion
Empagliflozin	25 mg	PO	Once daily	HF / T2DM / CKD
Apixaban	5 mg	PO	Twice daily	AF anticoagulation
Atorvastatin / Ezetimibe	80 mg / 10 mg	PO	Nightly / daily	ASCVD
Metformin	500 mg	PO	Once daily	T2DM (renal-adjusted)
Tamsulosin	0.4 mg	PO	Nightly	BPH
Pantoprazole	40 mg	PO	Once daily	GI protection
Denosumab	60 mg	SC	Every 6 months	Osteoporosis
Cholecalciferol / Calcium	2000 IU / 1000 mg	PO	Daily	Bone health
Sodium bicarbonate	650 mg	PO	Twice daily	CKD acidosis
Tiotropium	18 mcg	Inhaled	Once daily	COPD